

Executive Summary

UAC Care Transition Efficiency & Placement Outcome Analytics · Jan 2023 – Dec 2025

Purpose. Headcounts show how many children are in care, not whether the system is moving them to sponsors efficiently. This analysis measures process flow across the pipeline: CBP custody → HHS care → sponsor placement.

Key findings

- **The shrinking caseload is not the whole story.** The HHS census fell from 11,516 (Dec 2023) to about 2,000–2,500 in 2025, but mainly because intake dropped about 90%. The share of children discharged per report fell from 2.9% (2024) to 1.1% (2025): children now turn over about 2.6 times more slowly.
- **The bottleneck is discharge, not transfer.** Transfers out of CBP kept pace with intake in every year. In 2024, four sustained periods (16–61 consecutive reports) saw transfers outrun discharges, adding about 2,500 children to HHS care.
- **Placements became less stable in 2025.** The Outcome Stability Score fell from about 77 to 55 out of 100, with abrupt discharge collapses on 25 Feb and 23 Mar 2025 and two long low-discharge periods (Mar–Jul and Jul–Dec 2025).
- **Transfer efficiency is slipping.** The ratio of transfers to CBP custody fell from about 0.76–0.78 to 0.50 in 2025 (0.23 on the latest report).

KPI snapshot

KPI	2023	2024	2025
Transfer Efficiency Ratio	0.78	0.76	0.50
Discharge Effectiveness (% of HHS care per report)	3.33%	2.92%	1.14%
Backlog rate (children/report, + = building)	–131	+3.4	–12
Outcome Stability Score (0–100)	77.7	77.1	54.9

Recommended actions

- Adopt flow KPIs (discharge effectiveness, backlog rate, stability score) in routine reporting, with alert thresholds.
- Define a surge trigger: two weeks of net inflow to HHS above exits should prompt added case-management and sponsor-vetting capacity.
- Review the 2025 discharge slowdown using case-level data to identify whether sponsor verification, staffing or policy changes are the cause.
- Improve data: publish seven-day reports, all HHS entry routes, length of stay and sponsor category.

Caveats

Data are aggregate counts with irregular reporting (about five reports a week) and do not fully reconcile between stocks and flows; findings describe flow speed, not case quality or child safety, and causes are hypotheses for follow-up. Full methods, figures and limitations are in the research paper; a live dashboard allows any date range and threshold to be explored.